BARBADOS NATIONAL REGISTRY

Annual cardiovascular disease report

2025

Cardiovascular disease events and mortality in Barbados

The University of the West Indies | Cave Hill Campus

About this report

Annual cardiovascular disease report 2025

Cardiovascular disease events and mortality in Barbados

Purpose and scope

This report provides an annual overview of cardiovascular disease events and mortality in Barbados. It presents results on the burden of heart attacks and strokes in Barbados, together with selected sex and age summaries. The report is publically available, and is particularly aimed at supporting decision-making by government, hospitals, clinics, public-health teams, researchers and other partners.

The report presents a range of annual aggregated observations using the quality-controlled and approved BNR data releases. Estimates should be interpreted alongside the definitions, coverage notes and uncertainty information provided in the report. The report and the underlying data are available from the BNR Information Hub.

Report information

Report	**Annual cardiovascular disease report 2025**
Prepared by	Christina Howitt; Jacqueline Campbell; Ian Hambleton
Institution	Barbados National Registry | The University of the West Indies, Cave Hill Campus
Approved data releases	CVD events: cvd_2026_01 | CVD mortality: mort_2026_07
Online report	https://uwi-bnr.github.io/info-hub/
Suggested citation	Howitt C, Campbell J, Hambleton I for the Barbados National Registry. *Annual cardiovascular disease report 2025: cardiovascular disease events and mortality in Barbados.* The University of the West Indies, Cave Hill Campus, Bridgetown, Barbados.

Acknowledgements

The Barbados National Registry acknowledges the clinical, hospital, registry, data-management and public-health teams whose work supports the production of this surveillance report.

Further information

Definitions, ascertainment rules, mortality classifications, uncertainty measures, data availability and disclosure controls are described in the BNR Methods manual, available online. These methods, latest public report and associated data products are available at: https://uwi-bnr.github.io/info-hub/

Publication note: This edition is produced through the controlled BNR annual-report build, approval and publication workflow. The public version is the approved version deposited through that workflow.

Contents

2025 in brief

The latest complete year at a glance

CVD EVENTS

1,116

Primary national estimate

CVD EVENT RATE

229 per 100,000

Age-standardised | 95% CI 215-244

CVD DEATHS

368

Primary definition

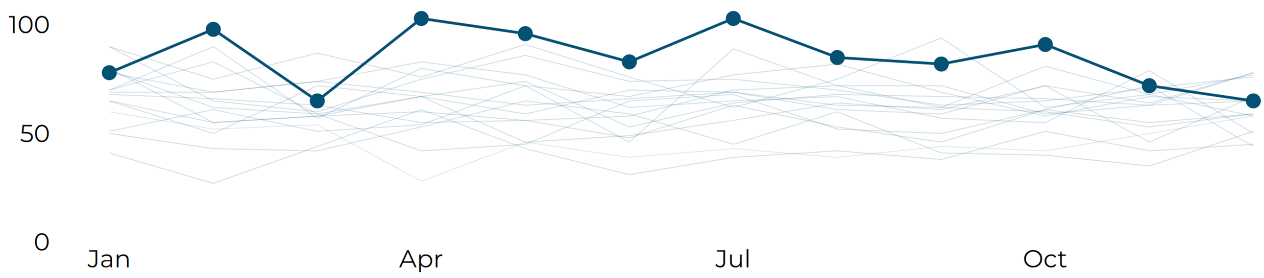
CVD MORTALITY RATE

76 per 100,000

Age-standardised | 95% CI 68-85

How the year unfolded

Monthly hospital-recorded CVD events in 2025. Unlike hospital events, national event estimates are updated annually because events captured only through death-record are added after each year closes.



What stood out in 2025?

01 There were 1,021 hospital-recorded CVD events in 2025, about 27% above the published previous-five-year average of 805.8. The Primary national estimate was 1,115.8 events after adding the estimated contribution from events identified through death records.

02 The Primary national age-standardised CVD event rate was higher in men than women: 288.8 compared with 184.0 per 100,000.

03 Stroke accounted for 67.9% of hospital-recorded CVD events in 2025.

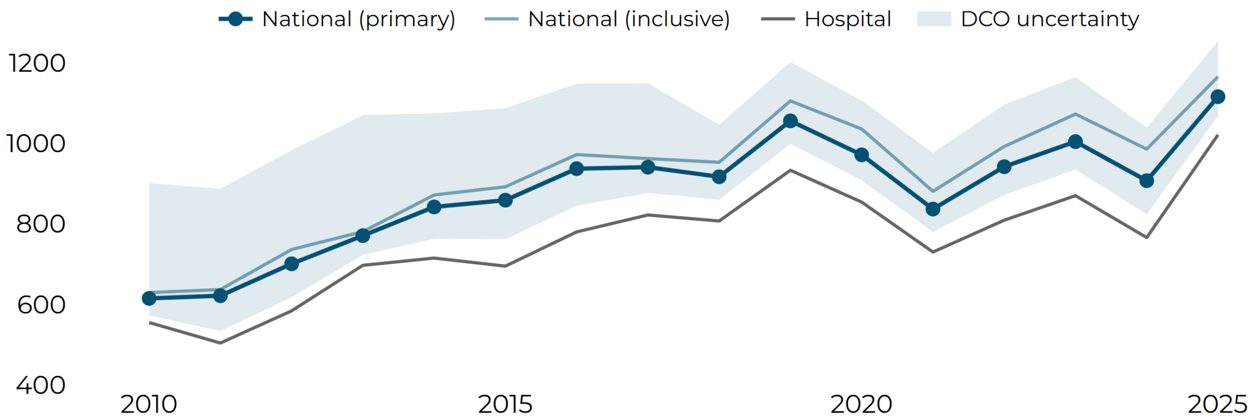
04 There were 368 Primary CVD deaths in 2025, about 7% below the previous-five-year mean of 394.2. The Inclusive definition counted 588 deaths, close to its recent five-year comparator, showing how strongly mortality totals depend on whether Possible deaths are included. Read more about our CVD death definitions in chapter 4|Methods.

1 | CVD events

CVD event counts

Hospital-recorded events and published national estimates across the complete annual series.

Hospital-recorded	Primary national	Inclusive national
1,021	1,116	1,165
events	events	events



Measure	2021	2022	2023	2024	2025
Hospital-recorded	730	809	870	766	1,021
Primary national	837	942	1,004	907	1,116
Inclusive national	881	992	1,073	986	1,165

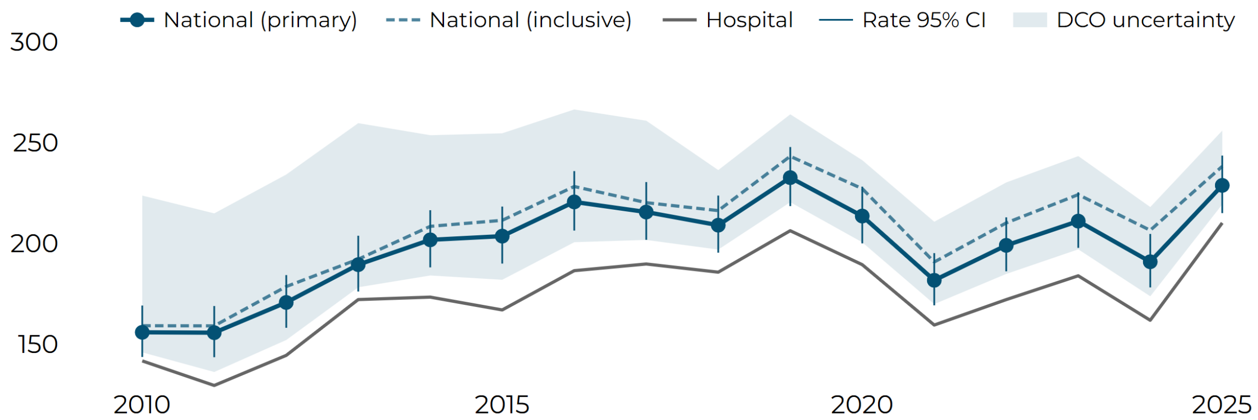
WHAT THIS MEANS

Hospital-recorded CVD events reached 1,021 in 2025: the highest annual count in the 2010 to 2025 series and 27% above the previous-five-year mean of 805.8. After including the estimated contribution from death-certificate-only events, the Primary national estimate was 1,115.8 and the Inclusive estimate was 1,165.4. All three measures point to an unusually high CVD event burden in 2025.

CVD event rates

Age-standardised rates per 100,000. Whiskers are published 95% statistical confidence intervals.

Hospital-recorded ASR	Primary national ASR	Inclusive national ASR
210	229	238
per 100,000	per 100,000	per 100,000



Latest five complete years

ASR per 100,000	2021	2022	2023	2024	2025
Hospital-recorded estimate	159.4	172.1	183.9	161.8	210.1
Hospital-recorded 95% CI	147.7 - 172.2	160.1 - 185.1	171.5 - 197.3	150.1 - 174.6	196.9 - 224.4
Primary national estimate	181.7	199.0	211.0	190.9	228.8
Primary national 95% CI	169.2 - 195.1	186.1 - 212.9	197.8 - 225.3	178.1 - 204.7	215.0 - 243.6
Inclusive national estimate	190.7	210.2	224.2	206.5	238.3
Inclusive national 95% CI	178.0 - 204.5	196.9 - 224.5	210.6 - 238.9	193.3 - 220.7	224.2 - 253.3

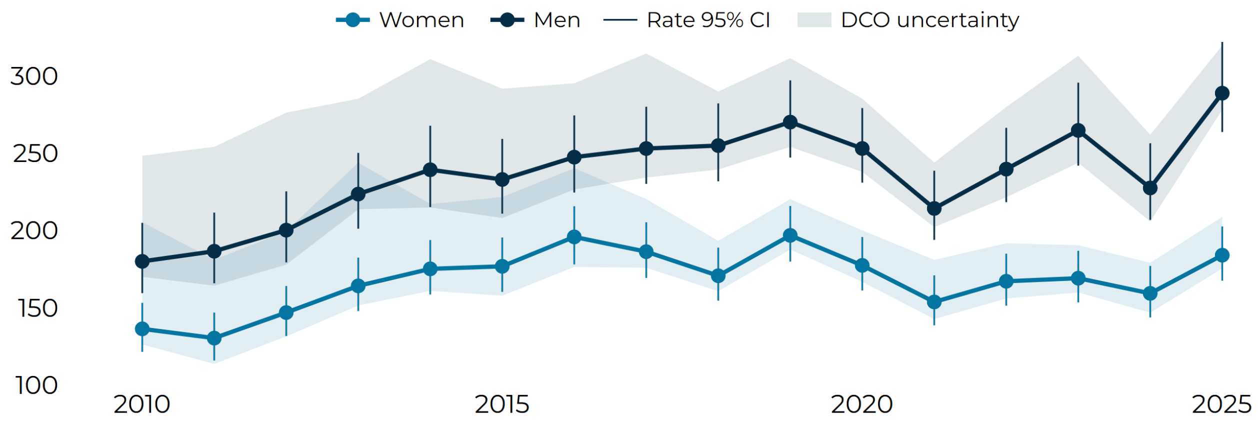
WHAT THIS MEANS

After allowing for differences in population age, the Primary national CVD event rate rose from 190.9 per 100,000 in 2024 to 228.8 in 2025, close to the series high of 232.6 in 2019. The 2025 hospital-recorded rate was 210.1 and the Inclusive national rate was 238.3. The three estimates give the same broad message of a high-rate year, while their separate confidence intervals and linkage ranges describe the uncertainty around the exact national level.

CVD events: women and men

Primary national age-standardised event rates per 100,000.

All sexes	Women	Men
229	184	289
per 100,000	per 100,000	per 100,000



Latest five complete years

Primary ASR per 100,000	2021	2022	2023	2024	2025
All sexes estimate	181.7	199.0	211.0	190.9	228.8
All sexes 95% CI	169.2 - 195.1	186.1 - 212.9	197.8 - 225.3	178.1 - 204.7	215.0 - 243.6
Women estimate	153.7	167.1	169.1	159.3	184.0
Women 95% CI	138.6 - 170.9	151.3 - 184.9	153.4 - 186.8	143.7 - 177.0	167.5 - 202.6
Men estimate	214.2	239.7	264.7	227.5	288.8
Men 95% CI	193.9 - 238.6	218.3 - 266.4	241.9 - 295.6	206.8 - 256.4	263.7 - 322.0

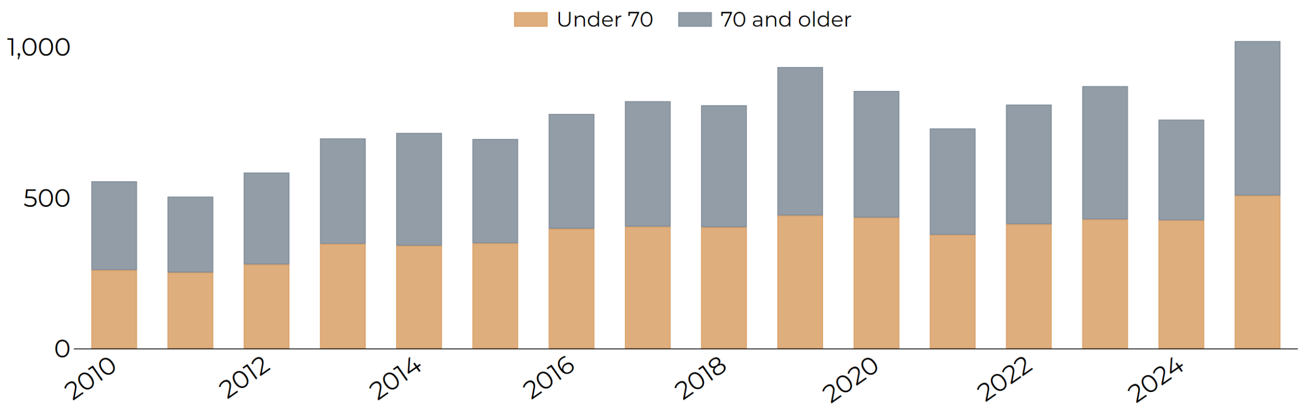
WHAT THIS MEANS

The 2025 Primary national age-standardised CVD event rate was 288.8 per 100,000 in men and 184.0 in women: about 57% higher in men. The male rate has been higher in every year of the 2010 to 2025 series, and the 2025 confidence intervals are clearly separated. This persistent difference is important for prevention and service planning; understanding its causes requires evidence on risk factors, diagnosis and access to care.

CVD events: age patterns

Hospital-recorded annual event counts by broad age group. These are composition counts, not age-specific rates.

All ages	Under 70	70 and older
1,021	508	511
events	events	events



Latest five complete years

Hospital-recorded events	2021	2022	2023	2024	2025
All ages	730	809	870	766	1,021
Under 70	378	413	429	426	508
70 and older	352	396	441	333	511

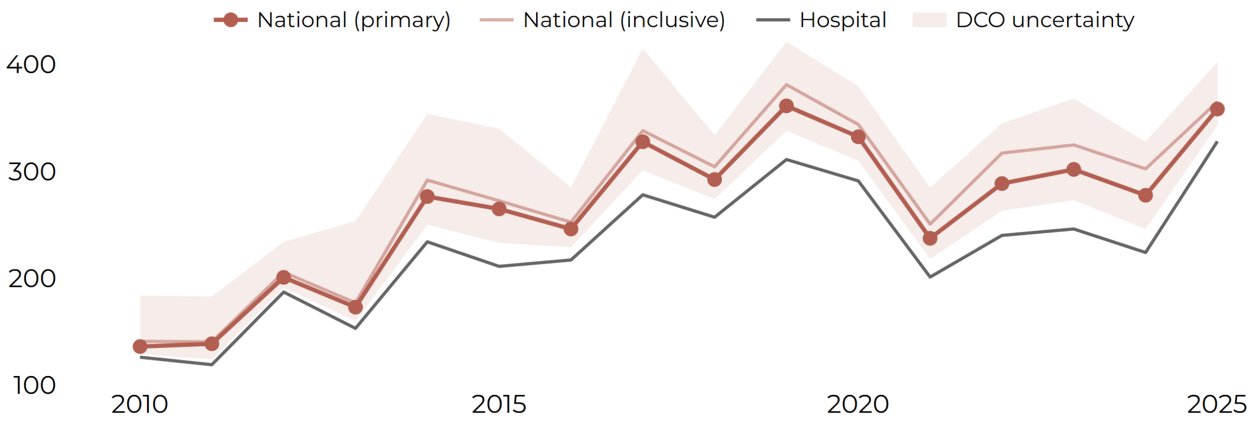
WHAT THIS MEANS

Hospital-recorded events were almost evenly divided by age in 2025: 508 among people under 70 and 511 among people aged 70 or older. Both groups were above their previous-five-year means of 416.2 and 388.2, so both contributed to the high overall count. These counts describe where recorded events occurred; age-specific rates, available online at the BNR Information Hub, are the appropriate measure for comparing population risk.

Heart event counts

Hospital-recorded events and published national estimates across the complete annual series.

Hospital-recorded	Primary national	Inclusive national
328	358	365
events	events	events



Measure	2021	2022	2023	2024	2025
Hospital-recorded	201	240	246	224	328
Primary national	237	289	302	278	358
Inclusive national	251	317	325	302	365

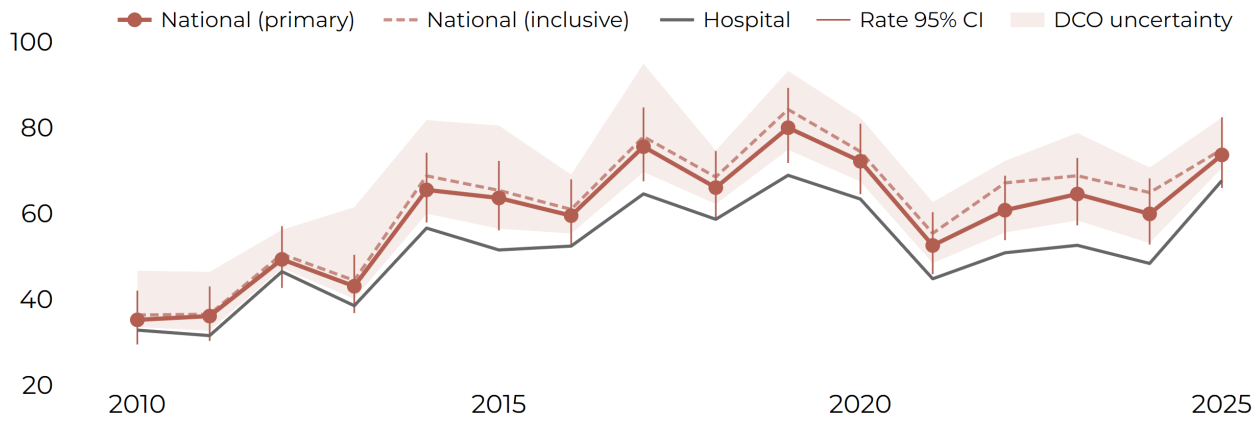
WHAT THIS MEANS

Hospital-recorded Heart events reached 328 in 2025: the highest annual count in the 2010 to 2025 series and 36% above the previous-five-year mean of 240.4. The Primary national estimate was 358.2 and the Inclusive estimate was 365.1. The closeness of the two national estimates strengthens the conclusion that 2025 was an unusually high year for Heart events.

Heart event rates

Age-standardised rates per 100,000. Whiskers are published 95% statistical confidence intervals.

Hospital-recorded ASR	Primary national ASR	Inclusive national ASR
68	74	75
per 100,000	per 100,000	per 100,000



Latest five complete years

ASR per 100,000	2021	2022	2023	2024	2025
Hospital-recorded estimate	44.8	50.8	52.6	48.3	67.7
Hospital-recorded 95% CI	38.6 - 52.0	44.5 - 58.3	46.0 - 60.2	41.9 - 55.9	60.3 - 76.2
Primary national estimate	52.5	60.8	64.5	59.9	73.7
Primary national 95% CI	45.9 - 60.3	53.8 - 68.8	57.2 - 72.9	52.7 - 68.2	65.9 - 82.4
Inclusive national estimate	55.4	67.1	68.8	64.9	74.9
Inclusive national 95% CI	48.5 - 63.3	59.7 - 75.6	61.3 - 77.4	57.4 - 73.4	67.2 - 83.8

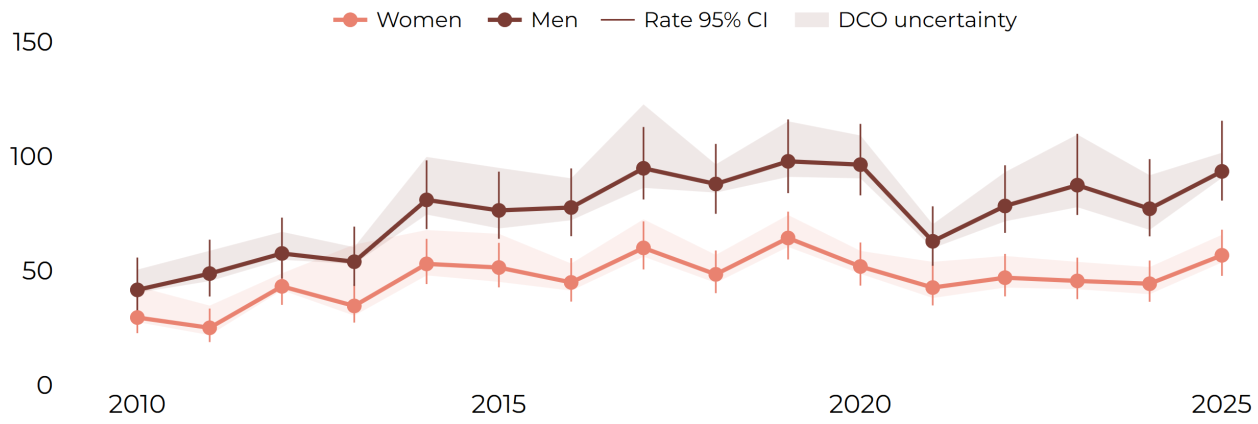
WHAT THIS MEANS

The 2025 Primary national Heart event rate was 73.7 per 100,000, up from 59.9 in 2024 and the highest point estimate since 2019. The hospital-recorded rate was 67.7 and the Inclusive national rate was 74.9. The 2024 and 2025 confidence intervals overlap slightly, so further years will help show whether this increase is sustained.

Heart events: women and men

Primary national age-standardised event rates per 100,000.

All sexes	Women	Men
74	**57**	**93**
per 100,000	per 100,000	per 100,000



Latest five complete years

Primary ASR per 100,000	2021	2022	2023	2024	2025
All sexes estimate	52.5	60.8	64.5	59.9	73.7
All sexes 95% CI	45.9 - 60.3	53.8 - 68.8	57.2 - 72.9	52.7 - 68.2	65.9 - 82.4
Women estimate	42.6	46.9	45.5	44.3	56.7
Women 95% CI	34.8 - 52.7	38.7 - 57.3	37.5 - 55.7	36.4 - 54.4	47.7 - 67.9
Men estimate	62.8	78.3	87.4	77.1	93.4
Men 95% CI	52.1 - 78.1	66.5 - 96.1	74.3 - 109.8	65.0 - 98.7	80.6 - 115.5

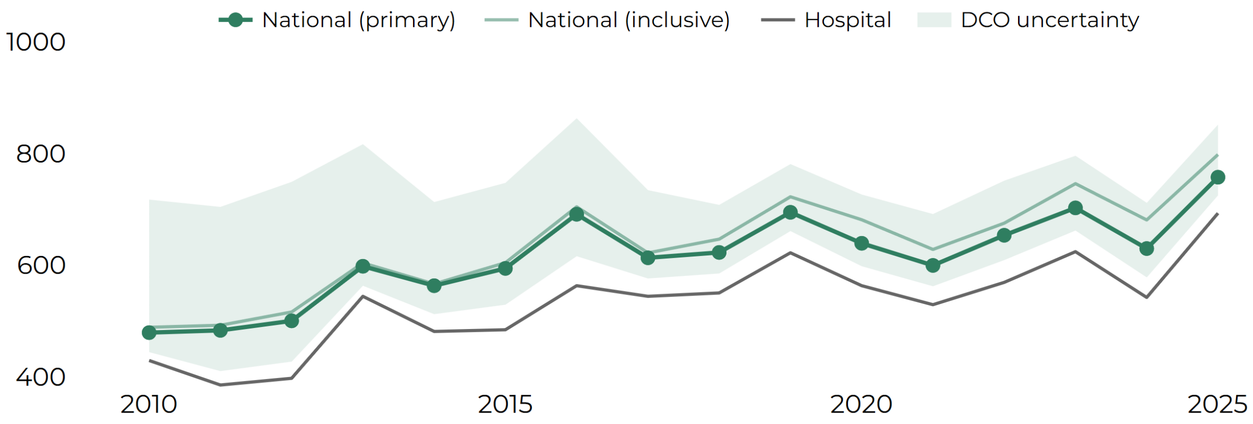
WHAT THIS MEANS

The 2025 Primary national Heart event rate was 93.4 per 100,000 in men and 56.7 in women: about 65% higher in men. The male rate has been higher throughout the 2010 to 2025 series, and the 2025 confidence intervals are clearly separated. This persistent pattern is important for prevention and service planning, while its causes require further evidence.

Stroke event counts

Hospital-recorded events and published national estimates across the complete annual series.

Hospital-recorded	Primary national	Inclusive national
693	**758**	**798**
events	events	events



Measure	2021	2022	2023	2024	2025
Hospital-recorded	529	569	624	542	693
Primary national	599	653	702	630	758
Inclusive national	628	675	746	681	798

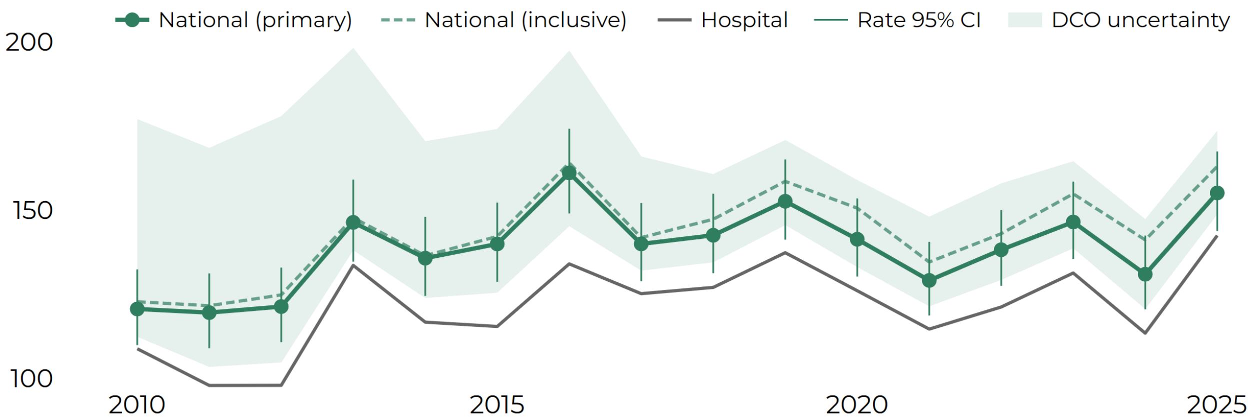
WHAT THIS MEANS

Hospital-recorded Stroke events reached 693 in 2025: the highest annual count in the 2010 to 2025 series and 23% above the previous-five-year mean of 565.4. The Primary national estimate was 757.5 and the Inclusive estimate was 798.2. As Stroke is the larger event group, this increase contributed most of the overall excess in CVD events above the recent mean.

Stroke event rates

Age-standardised rates per 100,000. Whiskers are published 95% statistical confidence intervals.

Hospital-recorded ASR	Primary national ASR	Inclusive national ASR
142	**155**	**163**
per 100,000	per 100,000	per 100,000



Latest five complete years

ASR per 100,000	2021	2022	2023	2024	2025
Hospital-recorded estimate	114.7	121.2	131.3	113.5	142.5
Hospital-recorded 95% CI	104.8 - 125.5	111.2 - 132.3	120.9 - 142.8	103.7 - 124.2	131.6 - 154.4
Primary national estimate	129.1	138.2	146.5	131.0	155.1
Primary national 95% CI	118.7 - 140.6	127.5 - 150.0	135.5 - 158.5	120.5 - 142.4	143.8 - 167.5
Inclusive national estimate	134.6	143.0	154.9	141.2	163.0
Inclusive national 95% CI	124.0 - 146.2	132.1 - 155.0	143.6 - 167.2	130.4 - 153.0	151.4 - 175.6

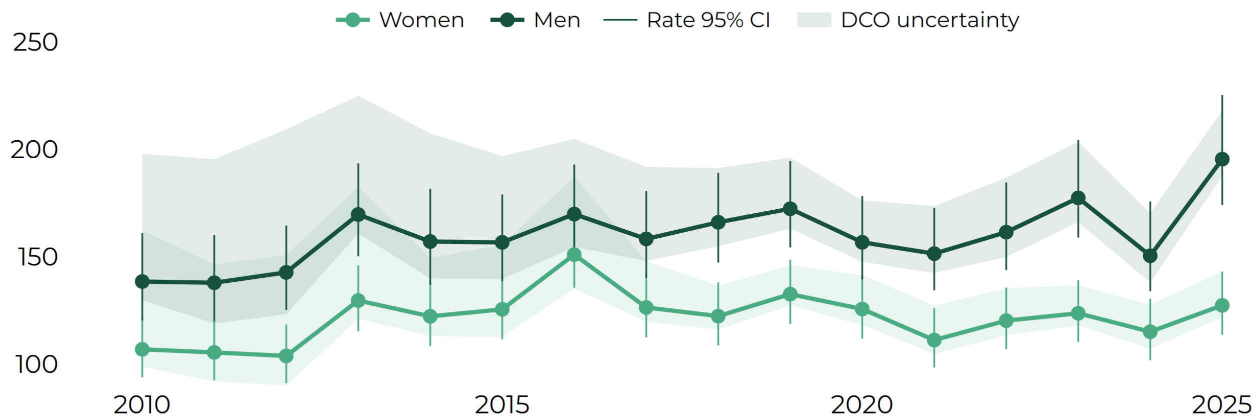
WHAT THIS MEANS

The 2025 Primary national Stroke event rate was 155.1 per 100,000, up from 131.0 in 2024 and the highest point estimate since 2016. The hospital-recorded rate was 142.5 and the Inclusive national rate was 163.0. The rise is visible across all three measures, supporting the broader finding of a high CVD event rate in 2025.

Stroke events: women and men

Primary national age-standardised event rates per 100,000.

All sexes	Women	Men
155	127	195
per 100,000	per 100,000	per 100,000



Latest five complete years

Primary ASR per 100,000	2021	2022	2023	2024	2025
All sexes estimate	129.1	138.2	146.5	131.0	155.1
All sexes 95% CI	118.7 - 140.6	127.5 - 150.0	135.5 - 158.5	120.5 - 142.4	143.8 - 167.5
Women estimate	111.1	120.2	123.6	115.0	127.3
Women 95% CI	98.4 - 126.0	106.9 - 135.6	110.3 - 139.0	101.7 - 130.4	113.6 - 143.1
Men estimate	151.4	161.4	177.4	150.4	195.4
Men 95% CI	134.3 - 172.6	143.7 - 184.6	158.9 - 204.3	133.9 - 175.7	174.0 - 225.2

WHAT THIS MEANS

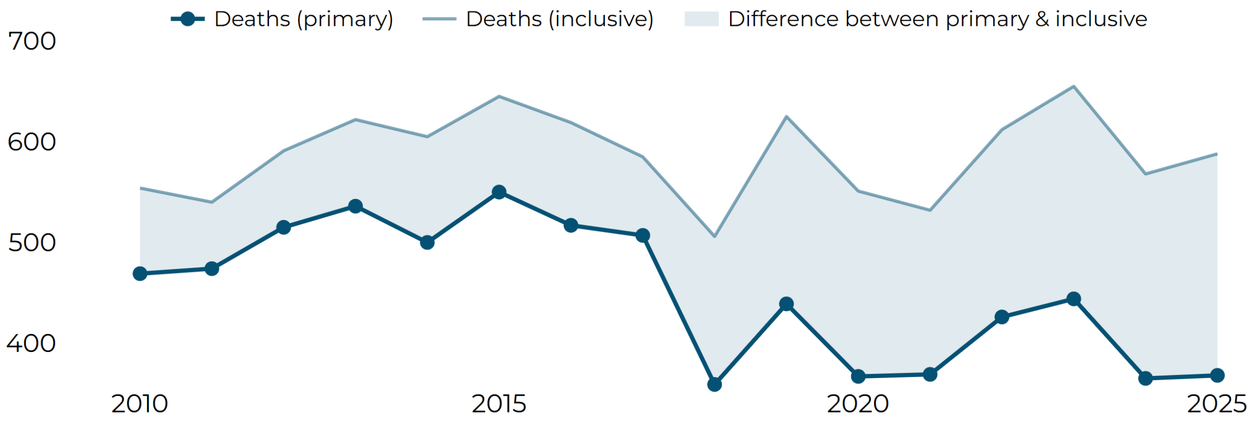
The 2025 Primary national Stroke event rate was 195.4 per 100,000 in men and 127.3 in women: about 54% higher in men. Rates rose for both sexes from 2024, with a larger increase among men. The 2025 confidence intervals are clearly separated, making this an important difference to monitor and address.

2 | CVD mortality

CVD deaths

Primary = Clear + Likely CVD deaths. Inclusive = Clear + Likely + Possible CVD deaths.

Primary deaths	Inclusive deaths
368	588
deaths	deaths



Latest five complete years

Deaths	2021	2022	2023	2024	2025
Primary	369	426	444	365	368
Inclusive	532	612	655	568	588

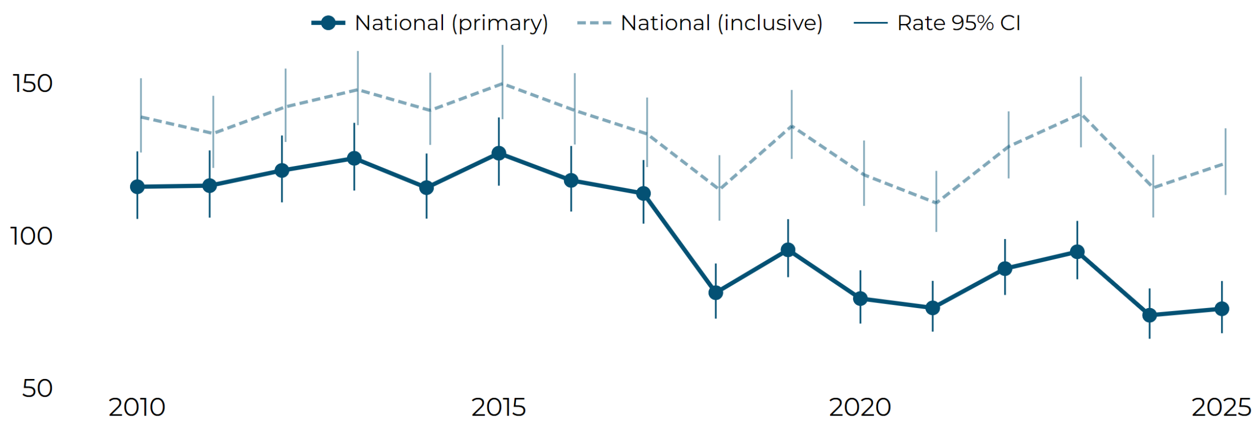
WHAT THIS MEANS

Primary CVD deaths numbered 368 in 2025, almost unchanged from 365 in 2024 and 7% below the previous-five-year mean of 394.2. The Inclusive count was 588, up from 568 in 2024 and close to its recent mean of 583.6. The difference of 220 deaths between Primary and Inclusive measures represents the Possible CVD class included in the broader definition and shows how strongly the total depends on cause-of-death classification uncertainty.

CVD mortality rates

Primary and Inclusive age-standardised mortality rates per 100,000. Whiskers are published 95% statistical confidence intervals.

Primary ASMR	Inclusive ASMR
76	124
per 100,000	per 100,000



Latest five complete years

ASMR per 100,000	2021	2022	2023	2024	2025
Primary estimate	76.3	89.2	94.7	73.9	76.0
Primary 95% CI	68.5 - 85.1	80.5 - 98.9	85.7 - 104.8	66.1 - 82.7	68.0 - 85.1
Inclusive estimate	110.7	129.2	140.0	115.7	123.7
Inclusive 95% CI	101.2 - 121.2	118.7 - 140.8	129.0 - 152.1	105.9 - 126.5	113.3 - 135.2

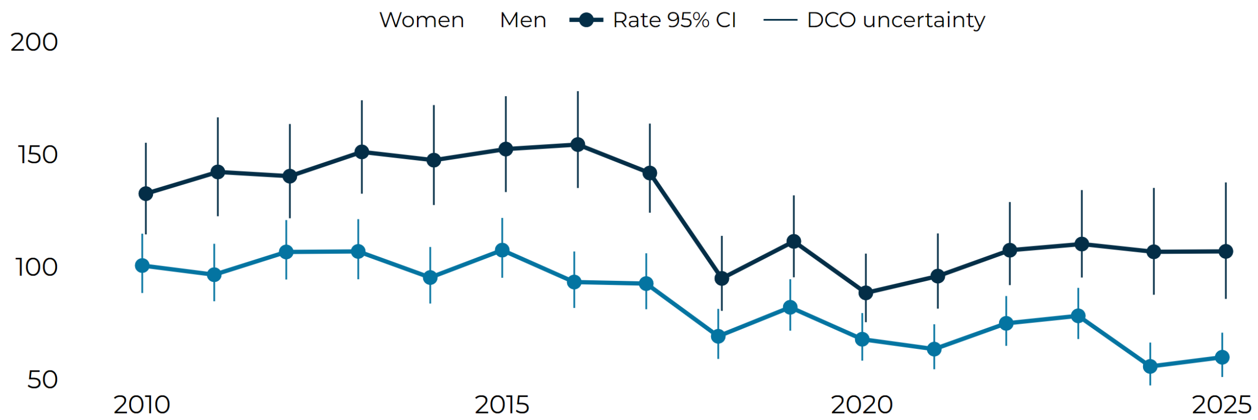
WHAT THIS MEANS

The Primary age-standardised CVD mortality rate was broadly stable, moving from 73.9 per 100,000 in 2024 to 76.0 in 2025. The Inclusive rate moved from 115.7 to 123.7, with overlapping confidence intervals across the two years. Each line has its own statistical confidence interval; the distance between the Primary and Inclusive lines shows the sensitivity of the result to including Possible CVD deaths.

CVD deaths: women and men

Primary age-standardised mortality rates per 100,000.

All sexes	Women	Men
76	60	107
per 100,000	per 100,000	per 100,000



Latest five complete years

Primary ASMR per 100,000	2021	2022	2023	2024	2025
All sexes estimate	76.3	89.2	94.7	73.9	76.0
All sexes 95% CI	68.5 - 85.1	80.5 - 98.9	85.7 - 104.8	66.1 - 82.7	68.0 - 85.1
Women estimate	63.4	74.8	78.2	55.7	59.7
Women 95% CI	54.4 - 74.4	64.8 - 87.0	67.8 - 90.6	47.2 - 66.3	50.9 - 70.7
Men estimate	95.8	107.4	110.1	106.7	106.8
Men 95% CI	81.3 - 114.8	91.8 - 128.8	95.2 - 134.1	87.5 - 135.1	85.7 - 137.5

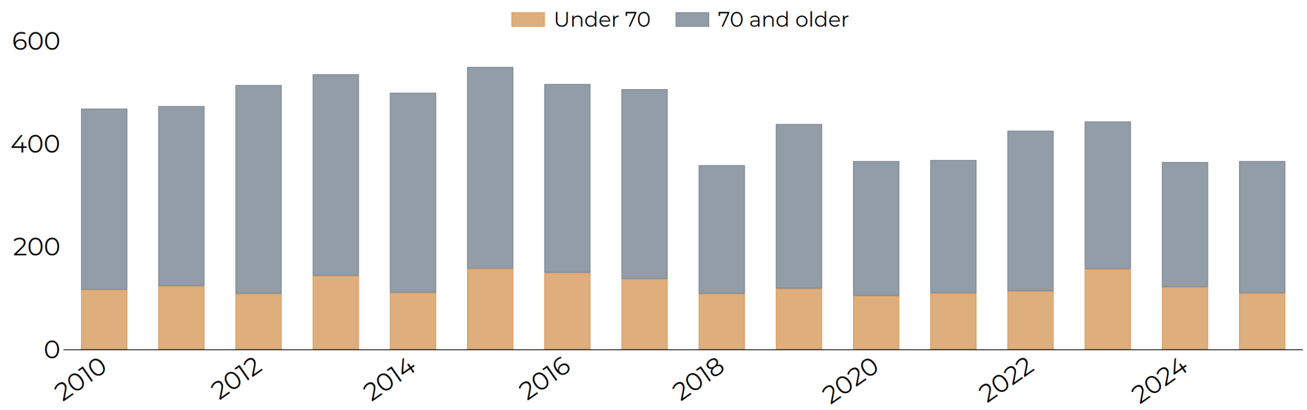
WHAT THIS MEANS

Primary CVD death counts were identical for women and men in 2025, at 184 each. After allowing for population age, the mortality rate was 106.8 per 100,000 in men and 59.7 in women: about 79% higher in men, with clearly separated confidence intervals. Age-standardised rates therefore reveal an important difference that the equal counts alone do not show.

CVD deaths: age patterns

Primary-definition annual death counts by broad age group. These are composition counts, not age-specific rates.

All ages	Under 70	70 and older
368	**110**	**257**
deaths	deaths	deaths



Latest five complete years

Primary deaths	2021	2022	2023	2024	2025
All ages	369	426	444	365	368
Under 70	110	114	157	122	110
70 and older	259	312	287	243	257

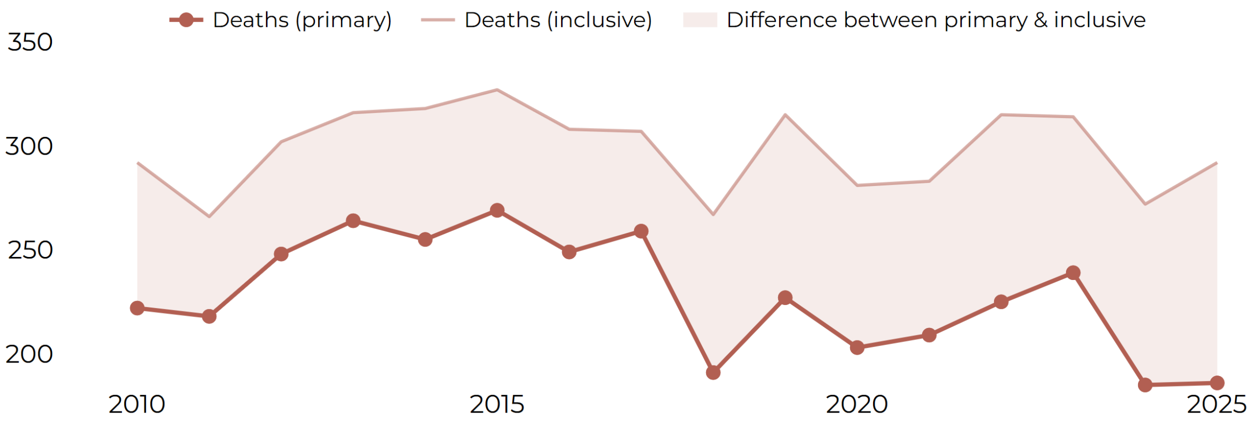
WHAT THIS MEANS

Of the 367 Primary CVD deaths with age recorded in 2025, 257 were among people aged 70 or older and 110 were among people under 70: 70% and 30% respectively. Both counts were below their previous-five-year means, so both age groups contributed to the lower overall total. One further Primary CVD death had no age classification and is excluded from these percentages.

Heart deaths

Primary = Clear + Likely CVD deaths. Inclusive = Clear + Likely + Possible CVD deaths.

Primary deaths	Inclusive deaths
186	**292**
deaths	deaths



Latest five complete years

Deaths	2021	2022	2023	2024	2025
Primary	209	225	239	185	186
Inclusive	283	315	314	272	292

WHAT THIS MEANS

Primary Heart deaths numbered 186 in 2025, almost unchanged from 185 in 2024 and 12% below the previous-five-year mean of 212.2. The Inclusive count was 292, up from 272 and almost equal to its recent mean of 293.0. The 106 deaths between the two 2025 estimates are Possible Heart deaths, showing the effect of the broader classification.

Heart mortality rates

Primary and Inclusive age-standardised mortality rates per 100,000. Whiskers are published 95% statistical confidence intervals.

Primary ASMR

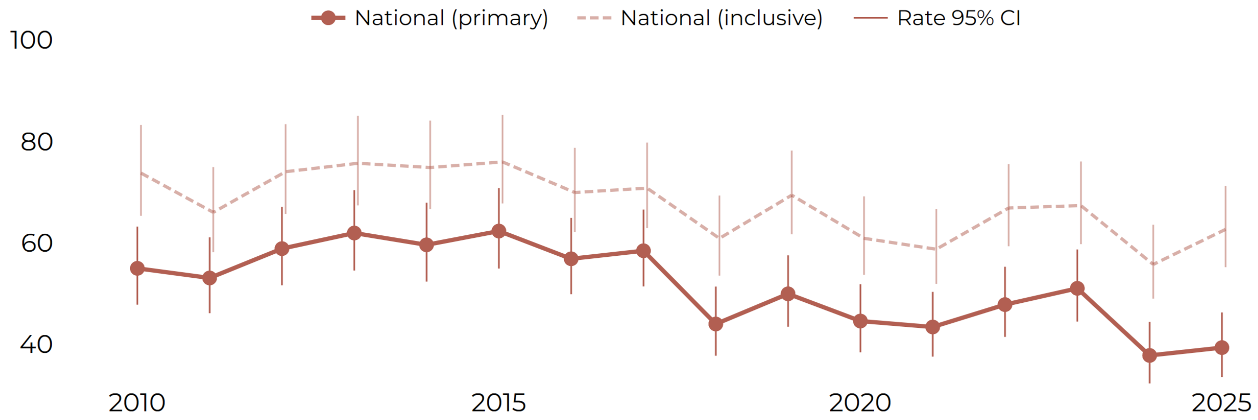
39

per 100,000

Inclusive ASMR

63

per 100,000



Latest five complete years

ASMR per 100,000	2021	2022	2023	2024	2025
Primary estimate	43.4	47.8	51.0	37.8	39.3
Primary 95% CI	37.5 - 50.3	41.4 - 55.3	44.4 - 58.7	32.2 - 44.4	33.5 - 46.2
Inclusive estimate	58.7	66.9	67.3	55.7	62.6
Inclusive 95% CI	51.9 - 66.6	59.3 - 75.5	59.7 - 76.0	49.0 - 63.6	55.2 - 71.2

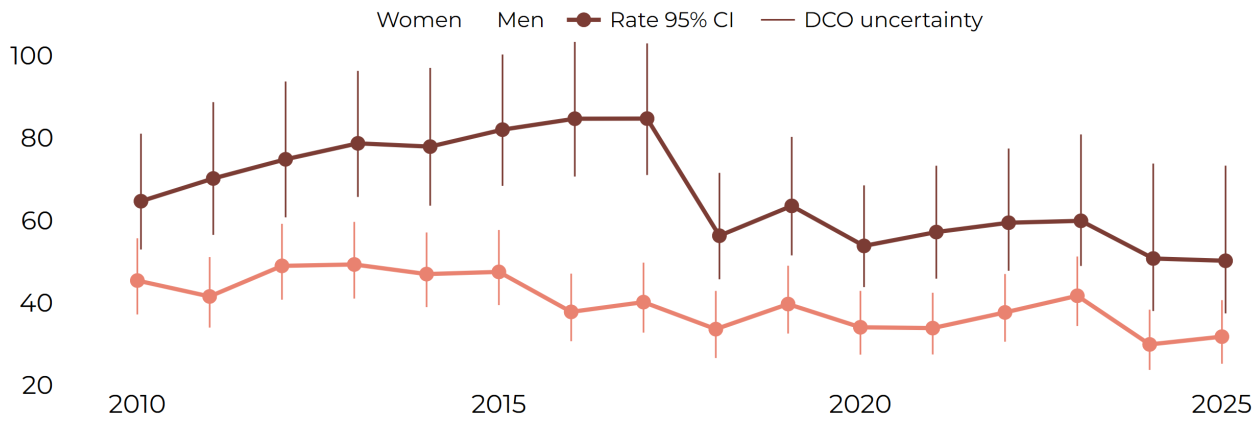
WHAT THIS MEANS

The Primary Heart mortality rate was broadly stable, moving from 37.8 per 100,000 in 2024 to 39.3 in 2025, and remained below the rates seen in 2021 to 2023. The Inclusive rate rose from 55.7 to 62.6, although its confidence intervals overlap across the two years. The distance between the lines shows the effect of including Possible Heart deaths.

Heart deaths: women and men

Primary age-standardised mortality rates per 100,000.

All sexes	Women	Men
39	**32**	**50**
per 100,000	per 100,000	per 100,000



Latest five complete years

Primary ASMR per 100,000	2021	2022	2023	2024	2025
All sexes estimate	43.4	47.8	51.0	37.8	39.3
All sexes 95% CI	37.5 - 50.3	41.4 - 55.3	44.4 - 58.7	32.2 - 44.4	33.5 - 46.2
Women estimate	33.8	37.6	41.7	29.9	31.7
Women 95% CI	27.4 - 42.4	30.5 - 47.0	34.3 - 51.2	23.7 - 38.3	25.2 - 40.6
Men estimate	57.2	59.4	59.9	50.7	50.2
Men 95% CI	45.8 - 73.3	47.7 - 77.4	48.9 - 80.9	38.0 - 73.8	37.4 - 73.3

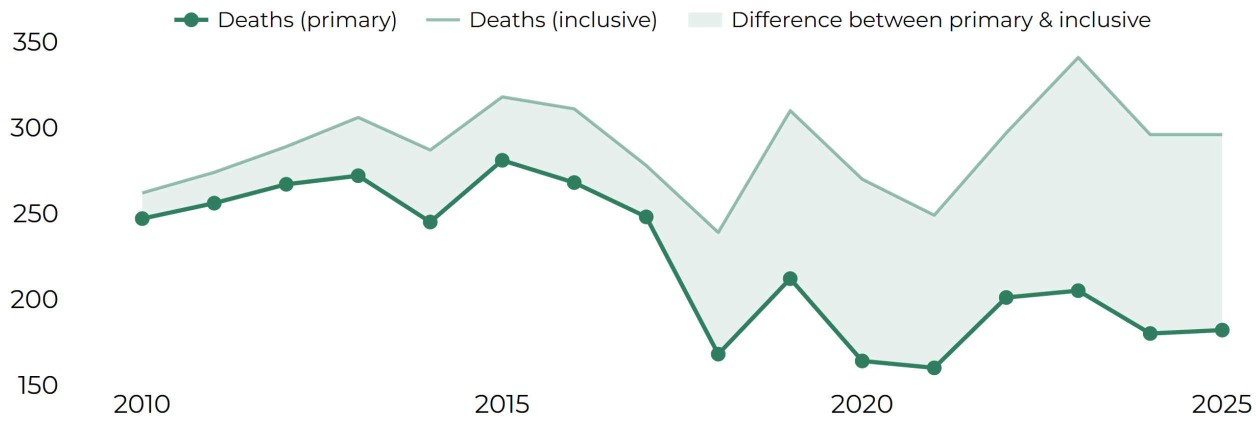
WHAT THIS MEANS

Primary Heart death counts were similar for women and men in 2025, at 92 and 94. The age-standardised rate was higher in men, at 50.2 per 100,000 compared with 31.7 in women, as it has been throughout the series. The 2025 confidence intervals overlap slightly, so the size of the difference is estimated with some uncertainty.

Stroke deaths

Primary = Clear + Likely CVD deaths. Inclusive = Clear + Likely + Possible CVD deaths.

Primary deaths	Inclusive deaths
182	296
deaths	deaths



Latest five complete years

Deaths	2021	2022	2023	2024	2025
Primary	160	201	205	180	182
Inclusive	249	297	341	296	296

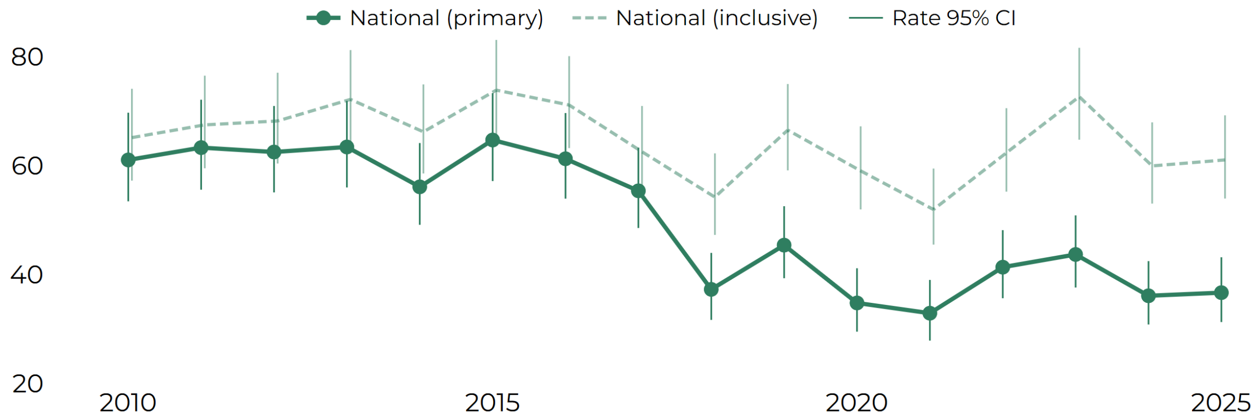
WHAT THIS MEANS

Primary Stroke deaths numbered 182 in 2025, close to 180 in 2024 and exactly equal to the previous-five-year mean of 182.0. The Inclusive count was unchanged at 296 and was also close to its recent mean of 290.6. The stable recent pattern sits alongside substantial classification sensitivity: Possible Stroke deaths make up 114 of the Inclusive total.

Stroke mortality rates

Primary and Inclusive age-standardised mortality rates per 100,000. Whiskers are published 95% statistical confidence intervals.

Primary ASMR	Inclusive ASMR
37	**61**
per 100,000	per 100,000



Latest five complete years

ASMR per 100,000	2021	2022	2023	2024	2025
Primary estimate	32.9	41.4	43.7	36.1	36.7
Primary 95% CI	27.9 - 39.0	35.7 - 48.2	37.6 - 50.9	30.8 - 42.5	31.3 - 43.2
Inclusive estimate	52.0	62.4	72.7	60.0	61.1
Inclusive 95% CI	45.5 - 59.5	55.2 - 70.6	64.8 - 81.7	53.1 - 68.0	54.0 - 69.3

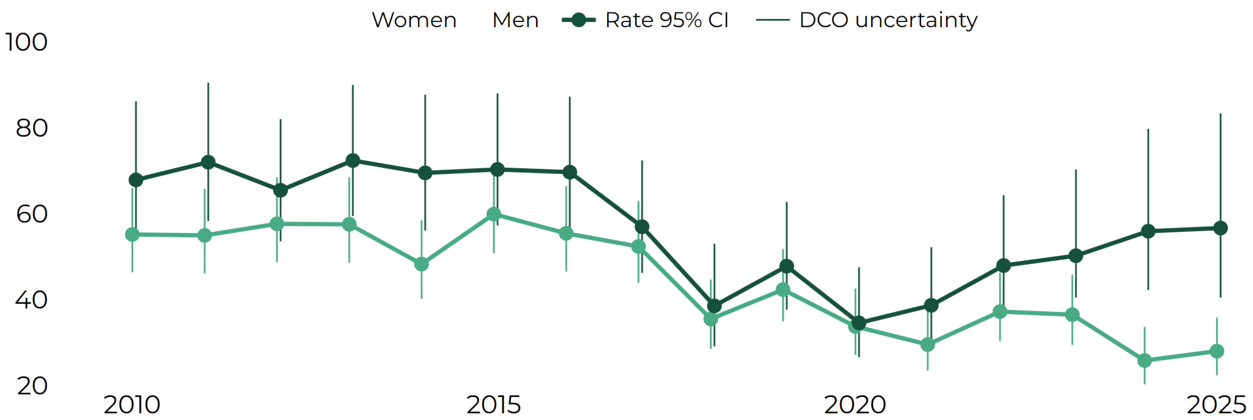
WHAT THIS MEANS

Stroke mortality rates were broadly stable between 2024 and 2025. The Primary rate moved from 36.1 to 36.7 per 100,000, while the Inclusive rate moved from 60.0 to 61.1; both pairs of confidence intervals overlap substantially. The continuing distance between the two lines shows the effect of including Possible Stroke deaths.

Stroke deaths: women and men

Primary age-standardised mortality rates per 100,000.

All sexes	Women	Men
37	**28**	**57**
per 100,000	per 100,000	per 100,000



Latest five complete years

Primary ASMR per 100,000	2021	2022	2023	2024	2025
All sexes estimate	32.9	41.4	43.7	36.1	36.7
All sexes 95% CI	27.9 - 39.0	35.7 - 48.2	37.6 - 50.9	30.8 - 42.5	31.3 - 43.2
Women estimate	29.5	37.2	36.5	25.8	28.0
Women 95% CI	23.5 - 37.8	30.3 - 46.2	29.5 - 45.7	20.2 - 33.6	22.4 - 35.8
Men estimate	38.7	47.9	50.2	55.9	56.7
Men 95% CI	30.0 - 52.2	38.1 - 64.3	40.5 - 70.3	42.2 - 79.8	40.4 - 83.4

WHAT THIS MEANS

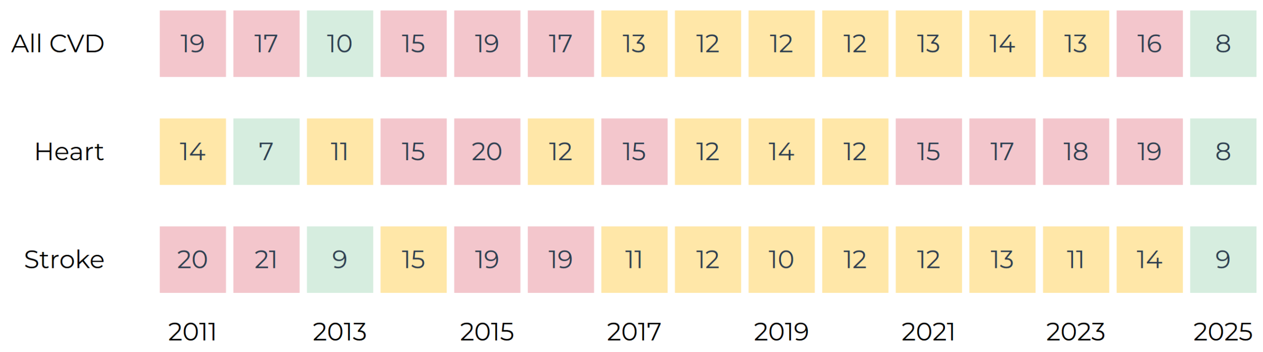
Primary Stroke death counts were also similar for women and men in 2025, at 92 and 90. After allowing for population age, the mortality rate was 56.7 per 100,000 in men and 28.0 in women: about twice as high in men, with separated confidence intervals. The persistent male-female difference is important for prevention and service planning.

Sources: BNR public CVD-event release cvd_2026_01; BNR public mortality release mort_2026_07.

3 | How complete is the picture?

Events identified through death records

Each tile gives the estimated additional DCO contribution as a percentage of the Primary national event estimate. The latest 15 complete years are shown. **Green** is lower reliance, amber moderate and red higher.

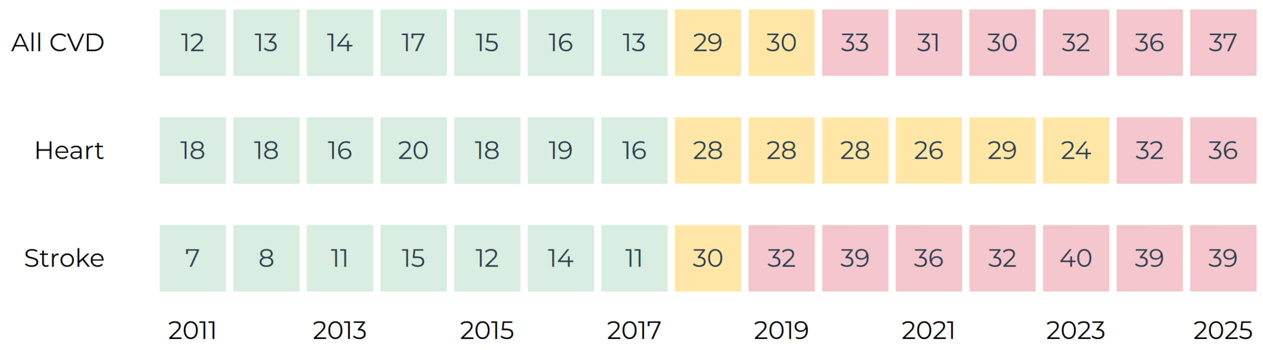


WHAT THIS MEANS

In 2025, the estimated additional DCO contribution was about 8.5% of the Primary national event estimate for All CVD, Heart and Stroke. The similarity across the three groups is useful context: death-record ascertainment contributes meaningfully to the national estimate, but it is not the dominant component of the 2025 event total.

Reliance on Possible deaths

Possible-only deaths as a percentage of the Inclusive annual mortality count. Green is lower reliance, amber moderate and red higher; the percentage remains the main information.



WHAT THIS MEANS

Possible-only deaths accounted for 37.4% of the 2025 Inclusive CVD total deaths. The corresponding proportions were also substantial for Heart and Stroke, at about 36% and 39%. These are sensitivity indicators for cause-of-death classification on death certificates. We use these sensitivity results in the absence of national underlying cause of death recording. Read more about our CVD death definitions in chapter 4|Methods.

4 | Methods

What this report measures

WHY THESE METHODS MATTER

National surveillance brings together information recorded for different purposes, at different times and in different settings. The BNR applies consistent clinical, residency, timing, linkage and quality rules so these records can support a coherent national picture. Each published result can be traced to an approved data release and is accompanied by clear information about its coverage, uncertainty and confidentiality.

Data used for this report

Series	Complete annual coverage	Public data release used
CVD events	2010 to 2025	cvd_2026_01 (2026, Jan)
CVD mortality	2010 to 2025	mort_2026_07 (2026, Jul)

Population, period and unit of analysis

This report describes eligible cardiovascular events and classified deaths among Barbados residents. Each event statistic counts eligible event episodes, so one person can contribute more than one event over time. Mortality statistics count deaths meeting the stated BNR evidence definition.

The main results cover complete calendar years from January to December. The monthly *How the year unfolded* chart provides the one within-year view. Monthly and quarterly event results describe hospital-recorded events; broader national estimates incorporating death records are completed annually.

A guide to the results

Result	What it describes	Main information source
Events	Eligible Heart and Stroke event episodes	Hospital records, extended annually with eligible death records
Deaths	Deaths meeting a stated BNR cardiovascular evidence definition	Death-certificate information
Population rates	Events or deaths in relation to the corresponding resident population	Approved releases and population estimates
Quality indicators	How much selected results depend on additional or less-certain information	Approved public-release summaries

How CVD events are identified

The BNR applies the same sequence of clinical, residency, timing and episode rules to every potential event. This turns information collected through routine care into a consistent surveillance series and supports fairer comparison over time.

COMPARING RESULTS OVER TIME

The complete series in this report can be compared consistently from 2010 onwards. The method was revised in 2025, so comparisons with earlier years should use the historical series reproduced in this report rather than estimates printed in reports from 2023 or earlier.

Heart, Stroke and All CVD

Each eligible event is assigned to Heart or Stroke under the maintained BNR rules. All CVD is the reporting aggregate formed by combining those two event groups.

Heart and Stroke event definitions

The clinical definition is the starting point. Residency, event date and episode rules then establish whether, when and how an eligible event contributes to the published series.

Reporting family	What it represents
Heart	Eligible acute myocardial infarction events, generally corresponding to ICD-10 I21-I22, confirmed under BNR clinical and validation rules.
Stroke	Eligible acute stroke events, generally corresponding to ICD-10 I60-I64, confirmed under BNR clinical and validation rules.

Decision	What the rule achieves
Disease definition	Confirms that the clinical evidence meets the approved Heart or Stroke definition.
Residence	Restricts the series to people resident in Barbados for at least six months of the relevant calendar year.
Event date	Places the event in the correct reporting period using the approved date hierarchy.
Event episode	Brings together records that describe the same clinical episode before counting.
Repeat event	Allows a later eligible event to contribute a new count when it occurs outside the 28-day event window.

Events and people

Each count represents an event episode. Records occurring within the 28-day event window are treated as part of the same episode. A later eligible event can contribute another count, so one person may contribute more than one event over time.

Hospital-recorded ascertainment

Hospital-recorded statistics use eligible events identified from hospital information. As Barbados's only tertiary hospital, the Queen Elizabeth Hospital provides an important view of serious recognised CVD events. Access to care, referral, diagnosis and recording all shape this view. The annual national reporting estimates extend it with eligible events identified through death records, as described next.

Extending the picture using death records

Some eligible cardiovascular events are identified through death information rather than hospital records. Bringing the two sources together broadens the national picture and requires careful classification and matching so that an event already represented in the hospital series is counted appropriately.

What is a DCO event?

A death-certificate-only (DCO) event is an eligible event identified from death information with no matched eligible hospital-recorded event. DCO-enhanced counts and rates are produced annually, after death-record ascertainment for the year is complete. Annual aggregation also supports safe reporting where monthly or quarterly DCO numbers may be small. Monthly and quarterly event views therefore describe hospital-recorded events.

How linkage is undertaken

Stage	What we do
1 Prepare	We prepare the hospital records and relevant death records for secure comparison.
2 Match	We compare records using approved combinations of names, dates and other identifying details, starting with the clearest matches.
3 Resolve	We accept clear matches and retain the uncertainty where the available information supports more than one interpretation.
4 Aggregate	We use the resolved links to produce anonymous aggregate event estimates for publication.

Three published views of CVD events

Published series	Information included	How to use it
Hospital-recorded	Eligible hospital events	Shows recognised serious events recorded through the hospital source. This is our conservative or lower estimate.
Primary national	Hospital events plus DCO contribution based on Clear + Likely mortality evidence	Provides the main BNR annual national estimate. This is our primary or central estimate.
Inclusive national	Hospital events plus DCO contribution that also includes Possible mortality evidence	Provides a broader estimate showing sensitivity to the wider evidence definition. This is our inclusive or upper estimate

During our matching process, we create 5 categories of matching: clear, likely, possible, mention only, no evidence. These uncertainty categories are described in detail on the next page.

Linkage uncertainty range

Where unresolved links could affect the national total, the public data provide three values. The lower value uses the more conservative interpretation, the central value is the main BNR estimate, and the upper value reflects the broader contribution supported by the approved linkage rules. Together they show how much the national estimate could change under the accepted interpretations of unresolved matches. Because the DCO contribution is estimated at aggregate level, national event estimates may contain decimal values before rounding for presentation.

WHEN READING THE REPORT | A wider linkage range means that unresolved matching has more influence on the national estimate. Statistical confidence intervals describe the precision of a rate and answer a different question.

How CVD deaths are classified

BNR mortality surveillance uses death-certificate information to create a consistent cardiovascular classification for routine surveillance. A formally coded national underlying cause of death is not currently available to the Registry for this purpose. Because certificates are received in paper form, the BNR first transcribes them into a standard electronic format on behalf of the Government, allowing every certificate to be assessed consistently.

How the certificate is interpreted

A cardiovascular term is interpreted in the context of the full certificate before a BNR classification is assigned. We consider the wording, whether it appears in Part I or Part II, the order of conditions in Part I, relevant mortality-coding principles and the strength of the supporting evidence. Part I records the sequence leading directly to death, while Part II records other contributing conditions. These details provide the context for assigning each death to one of five evidence classes.

Evidence classes

Class	What the evidence means	Use in published definitions
Clear	Strong evidence for the relevant cardiovascular mortality group	Primary and Inclusive
Likely	Good cardiovascular evidence with some remaining uncertainty	Primary and Inclusive
Possible	Plausible cardiovascular evidence with material uncertainty	Inclusive
Mention only	Relevant wording recorded as contextual evidence	Classification recorded. Not included in estimates.
No evidence	The approved rules identify no qualifying cardiovascular evidence	Classification recorded. Not included in estimates.

Heart, Stroke and All CVD

Heart includes qualifying ischaemic and coronary heart-disease evidence, including certificates that use terms other than 'heart attack'. Stroke includes qualifying acute ischaemic stroke and non-traumatic cerebral haemorrhage patterns. Each qualifying death is resolved to Heart or Stroke under the maintained hierarchy. All CVD is the reporting aggregate formed by combining those two groups.

Primary and Inclusive definitions

Primary combines Clear and Likely evidence and is the main BNR mortality series. Inclusive contains the Primary series and additionally includes Possible cardiovascular deaths. The distance between them shows how sensitive the reported burden is to including this less-certain evidence.

Relation to official cause-of-death statistics

Official cause-of-death statistics use the full international process for selecting and coding an ICD-10 underlying cause. The BNR uses a maintained death-certificate evidence classification to provide consistent and timely cardiovascular surveillance. The two series serve related but distinct purposes, so comparisons should take their different methods into account.

WHEN READING THE REPORT | Read Primary and Inclusive as alternative views of the same mortality burden. Primary is the main series; Inclusive shows how the result changes when Possible cardiovascular deaths are included.

How this report presents CVD

The report uses four complementary ways to describe CVD burden. Each answers a different question, so the measure and its denominator should always be read together.

Measure	Question it answers	Best used for
Count	How many eligible events or classified deaths were recorded?	Describing the volume of events or deaths
Percentage	What share of a defined total belongs to a particular group?	Describing the composition of that total
Crude rate	How frequent was the outcome in the population during that year?	Describing the population's observed experience
Age-standardised rate	How do rates compare after allowing for differences in age structure?	Comparing years, sexes or populations more fairly

Counts and percentages

A count reports the number of eligible event episodes or classified deaths. A percentage shows how a stated total is distributed across groups. For example, the percentage of registered events occurring before age 70 describes the age pattern among those events; it represents a share of the event total rather than a person's chance of developing CVD.

Crude and Age-standardised rates

A crude rate relates the number of events or deaths to the resident population, usually expressed per 100,000 people. It describes the population’s observed experience and is influenced by its age structure. An age-standardised rate applies age-specific rates to a common standard population, reducing the effect of differences in age structure. It is therefore better suited to comparisons across years, sexes or populations, but is a comparative measure rather than the population’s actual observed rate. We consider rates again on the next page.

Why denominators matter

The denominator defines the population or total to which a result refers. Counts have no population denominator. Percentages use the eligible total named with the result. Population rates pair events or deaths with the resident population for the same year and, where relevant, the same sex and age group.

Population rates and fair comparisons

Why use a population rate?

A count describes volume. A population rate relates that count to the number of residents who could contribute to it, allowing years or groups of different sizes to be compared. Event and mortality rates in this report are presented per 100,000 residents.

Crude and age-standardised rates

Rate	What it describes	How to use it
Crude	The observed number of events or deaths in relation to the matching resident population	Describes the population's actual experience during that year
Age-standardised	The rate after applying the same standard age distribution to every year or group	Supports fairer comparison where population age structures differ

Matching the population to the result

Every event or death numerator is paired with the Barbados resident population for the same year. Rates for women and men use the corresponding sex-specific population. Age-specific rates use the population in the stated age group. The report uses United Nations World Population Prospects, 2024 edition, for the population estimates.

Age standardisation

Cardiovascular disease is strongly related to age, and the age structure of a population can change over time. Direct age standardisation applies the WHO World Standard Population 2000-2025 to every comparison. This gives each year or group the same reference age structure, allowing differences in rates to be interpreted more fairly.

TECHNICAL DETAIL | Full formulas, denominator specifications and standard-population information are provided in the online BNR Methods manual: https://uwi-bnr.github.io/info-hub/methods/

Understanding uncertainty and time

Every surveillance estimate contains some uncertainty. In this report, uncertainty can arise when a rate is calculated from observed numbers, when hospital and death records are linked, and when the cardiovascular evidence on a death certificate is classified. We show each separately because each answers a different question about the result.

Source of uncertainty	The question it answers	How it appears
Statistical precision	How precisely has the rate been estimated from the observed number of events or deaths?	95% confidence interval
Linkage uncertainty	How much could unresolved matching change the annual national event estimate?	Lower, central and upper linkage values
Classification sensitivity	How much does the mortality result change when Possible cardiovascular deaths are included?	Primary and Inclusive results

How to read the three forms

A narrower confidence interval indicates a more precise rate estimate. A wider linkage range shows that unresolved matching has more influence on the national event estimate. A larger difference between Primary and Inclusive mortality shows greater sensitivity to the classification of less-certain death-certificate evidence.

Statistical confidence intervals

The 95% confidence interval shows the statistical precision of a published rate. Wider intervals commonly occur when the underlying number of events or deaths is small. Crude-rate intervals use the exact Poisson (Garwood) method where specified, and directly age-standardised rates use the Fay-Feuer gamma method.

Comparisons over time

The previous-five-year mean is the average for the five complete calendar years immediately before the year being reported. It provides recent historical context for the current result. Longer-term charts show the full internally comparable series and help readers see whether a recent change forms part of a sustained pattern.

WHEN READING THE REPORT | Read each form of uncertainty beside the estimate it describes. Together they show statistical precision, the influence of unresolved linkage and sensitivity to mortality classification.

Data quality and data confidentiality

Data quality metrics in Chapter 3

The indicators we present in Chapter 3 show how two important parts of the method contribute to the published results: extending hospital-recorded events with eligible death records (the DCOs), and including Possible cardiovascular deaths in the broader mortality definition. They make the influence of these choices visible alongside the disease estimates.

DCO contribution	The percentage of the Primary national event estimate contributed by eligible death-certificate-only events.
Possible-death reliance	The percentage of the Inclusive mortality total contributed by deaths in the Possible evidence class.

Data completeness

Where completeness is reported, it shows the proportion of eligible records containing the information required for that measure. Information that does not apply to a record is excluded from the expected total. Reading completeness alongside the results helps distinguish changes in recording from changes in CVD burden.

Protecting confidentiality

The BNR publishes aggregate totals, rates and summaries. Exact counts from 1 to 5 are suppressed, and additional values are protected where totals, calculated results or comparisons across releases could reveal a small count. An asterisk marks a protected result and means that its value remains confidential. Every proposed public release receives human review before approval.

TECHNICAL DETAIL | A data completeness report and the complete disclosure-control method is documented in the online BNR Methods manual: https://uwi-bnr.github.io/info-hub/methods/

Before publication

The underlying data are prepared, checked, reviewed and approved before publication. The annual report links to specific approved BNR data releases, preserving a traceable connection between the published data and every result shown here.

Responsible interpretation

Changes over time may reflect genuine shifts in population health, but they can also arise from changes in access to care, diagnosis, recording, data completeness or population estimates. Results should therefore be compared on a consistent basis: events should not be treated as numbers of people, and hospital-recorded results should be distinguished from national estimates. Each form of uncertainty should be considered alongside the estimate to which it relates. Surveillance findings can guide decisions and identify areas for closer investigation, but they do not by themselves explain why a pattern has occurred.

WHY THE RESULTS CAN BE USED WITH CONFIDENCE

These results are produced through a clear, reproducible and carefully governed process. Consistent definitions are applied across complementary data sources; uncertainty and known limitations are made explicit; every result can be traced to a named public release; and confidentiality checks and human approval precede publication. No surveillance system is free from limitation, but this approach provides a transparent and dependable evidence base for public health decisions.

Further information

TEMPORARY WEB ADDRESSES - VERIFY BEFORE PUBLICATION Public Methods manual: https://uwi-bnr.github.io/info-hub/methods/Operations manual: https://uwi-bnr.github.io/info-hub/operations/ Technical manual: https://uwi-bnr.github.io/info-hub/technical/

Special chapter | 2025

The BNR Refit

Building a faster, safer and more transparent cardiovascular reporting system

Building on a strong national resource

For many years, the Barbados National Registry has provided an important national record of cardiovascular disease. The BNR Refit strengthens that foundation by connecting case capture, analysis and public reporting through one clearly governed route. Its purpose is to provide government, hospitals, clinics and public-health partners with evidence that is more timely, reproducible and easier to understand.

The refit brings together three established software platforms, each with a clear and distinct role. REDCap supports the structured management of registry data. Maintained Stata workflows apply agreed definitions, carry out analytical checks and produce the statistics. Quarto provides the publishing framework for the BNR Information Hub, where approved results are presented through webpages, dashboards and reports. Keeping these roles separate reduces unnecessary copying and reworking, lowers the risk of inconsistencies, and makes it easier to trace every published result back to its source data, method and approval.

Three questions guided the review

We first conducted a full review of the original BNR processes. The review was shaped by three practical questions. Together, they tested whether published results could be traced to an approved source, produced consistently wherever they appear, and made available promptly without compromising confidentiality. Review findings are presented on the next page.

TRACEABILITY

Can every published result be traced to an approved data release?

CONSISTENCY

Are the same definitions and calculations used wherever a result appears?

TIMELINESS AND SAFETY

Can checked information be published quickly while protecting individuals?

Four connected areas

The subsequent BNR refit has been guided by the findings of this review. Work has been organised across four connected areas, covering the full route from managed data to public information. Each area supports the others: dependable reporting relies on sound data, consistent methods, clear governance and an effective way to share approved results.

DATA FOUNDATION

Managed REDCap data, a controlled historical data record and named source releases

DEFINITIONS AND METHODS

Consistent event, mortality, DCO, statistical and uncertainty methods

ASSURANCE AND GOVERNANCE

Automated data quality checks, human approval, disclosure protection and versioned publication

PUBLIC REPORTING

Connected dashboards, monthly updates, annual reports and one-off reports

THE REFIT PERIOD

The main refit programme runs from April 2026 to March 2027. As of December 2026, core data, event, mortality and dashboard workflows are operating. Report workflows, final testing, manuals and handover continue into early 2027; later analytical modules remain future work.

What the review identified

The BNR process review followed the complete pathway from case capture through data preparation, analysis and reporting. Ten connected findings summarise the issues that shaped this BNR refit. In the table below, we use a short reference code for our 10 issues, so that every finding can be matched directly to the subsequent improvements, detailed on the next page.

Ref.	Area	What the review identified	Why it mattered
D1	**Data foundation**	Historical datasets, identifiers and analytical records required reconciliation	The authoritative record and the source behind a result needed to be easier to establish
D2	**Data foundation**	Database validation, core-variable protection, cleaning and metadata needed greater standardisation	Missing or inconsistent information could otherwise travel into later stages and delay reporting
M1	**Definitions and methods**	Eligibility, case-finding and event-inclusion rules had varied over time	Changing rules reduce the comparability of trends
M2	**Definitions and methods**	Duplicate and repeat-event controls needed consistent application	Duplicate records can inflate counts and obscure genuine repeat events
M3	**Definitions and methods**	Mortality and DCO evidence required a structured, reproducible classification and linkage method	National estimates and their uncertainty needed to be transparent
M4	**Definitions and methods**	Data preparation, analysis and report production had developed within overlapping processes	Each cycle required avoidable manual handling and was harder to reproduce
G1	**Assurance and governance**	Dataset sign-off, release decisions, roles and version control needed one visible route	Unclear responsibility makes approval and recovery harder to audit
G2	**Assurance and governance**	Quality monitoring and protection of small public figures needed to be systematic	Problems and disclosure risks should be identified before publication
R1	**Public reporting**	Reporting products were produced through separate, relatively slow processes	Useful information could remain unavailable after data had been collected and checked
R2	**Public reporting**	Methods, operating knowledge and variable definitions needed sustainable open documentation	The system should be transparent and maintainable by a small BNR team

The original audit also identified longer-term opportunities including abstraction automation, additional condition modules, research analytics, wider technical governance and periodic independent review. These remain future options and are not presented here as completed parts of the core refit.

How each finding became an improvement

The references and ordering below match the findings table on the previous page. The delivery progress is presented in greater detail on the next page.

Ref.	Issue identified	What the refit introduced	Direct benefit	Delivery progress
D1	**Records required reconciliation**	A reconciled historical foundation, documented lineage and named source releases	Every reporting cycle starts from an identifiable approved source	Operating by Dec 2026
D2	**Data controls needed standardisation**	Managed REDCap database structures, validation checks, a protected core variable set and maintained metadata	Quality issues can be found earlier, before analytical production	Operating by Dec 2026
M1	**Inclusion rules varied over time**	Maintained eligibility, event-definition and case-inclusion rules	Trends and products use consistent definitions	Operating by Dec 2026
M2	**Duplicate controls were inconsistent**	Standard duplicate checks and explicit repeat-event rules before counting	Counts are less vulnerable to duplicate representations	Operating by Dec 2026
M3	**Mortality methods needed structure**	Reproducible mortality classification, DCO identification, linkage and uncertainty methods	Primary and Inclusive national estimates can be interpreted transparently	Operating by Dec 2026
M4	**Analytical processes overlapped**	Separate, modular Stata workflows for preparation, calculation, review and publication	Fewer manual changes and a more reproducible analytical cycle	Operating; testing to Mar 2027
G1	**Approval routes needed clarification**	Fixed monthly data releases, full versioning, recorded human approval and controlled publication	Release decisions and recovery are visible and auditable	Operating by Dec 2026
G2	**Quality and disclosure checks needed consistency**	Automated quality checks, small-number suppression and whole-output disclosure review	Public information is checked and protected consistently	Operating; testing to Mar 2027
R1	**Reporting was slow and separate**	Approved data releases feeding dashboards, monthly updates, annual reports and one-off reports	New information reaches users without rebuilding every product independently	Dashboards operating; annual Jan and one-off Feb 2027
R2	**Documentation needed strengthening**	Linked Technical, Operations and public Methods manuals in open formats	Methods are transparent and the system is easier to maintain and hand over	Completion by Mar 2027

The overall redesign is structural, from the ground-up. The new system replaces the repeated manual preparation of separate (hard-copy only) reports with one controlled route from managed data to approved public information.

From approved data to public information

The refit replaces a sequence of largely separate analytical and reporting tasks with a controlled digital route. Once data have been managed, checked and approved, the same data release can supply public dashboards and linked outputs without waiting for each product to be rebuilt independently.

A controlled publication route

1 MANAGE	REDCap holds managed registry data, supported by defined checks and metadata.
2 RELEASE	A monthly named data release defines the approved source for the reporting cycle.
3 COMPUTE	Readable, modular Stata workflows create the agreed measures and outputs.
4 CHECK AND PROTECT	Automated validation checks results and withholds very small public figures; the complete output is then reviewed for disclosure risk.
5 APPROVE	An authorised reviewer confirms analytical accuracy, provides interpretation and approves readiness.
6 PUBLISH	Only approved, versioned data releases and associated publications move to the public-access Information Hub.

What digital transformation changes

FASTER

Approved information can move rapidly into connected public products.

CONSISTENT

Dashboards, tables and reports use the same definitions and data releases.

SAFER

Disclosure protection and human review are built into publication.

TRACEABLE

Each result is linked to a named data release, maintained code and documented method.

A RELIABLE HISTORICAL FOUNDATION

The 2009-2023 record was carefully reconstructed and reconciled to provide a documented, traceable historical foundation for the new system. This preserves the value of more than a decade of surveillance while making the origins and limitations of the data clearer. Some older records contain fewer identifiers for matching information across data sources than more recent records. Where this limits linkage, the resulting uncertainty is reported rather than hidden.

Special chapter | The BNR Refit

WHAT HAS CHANGED

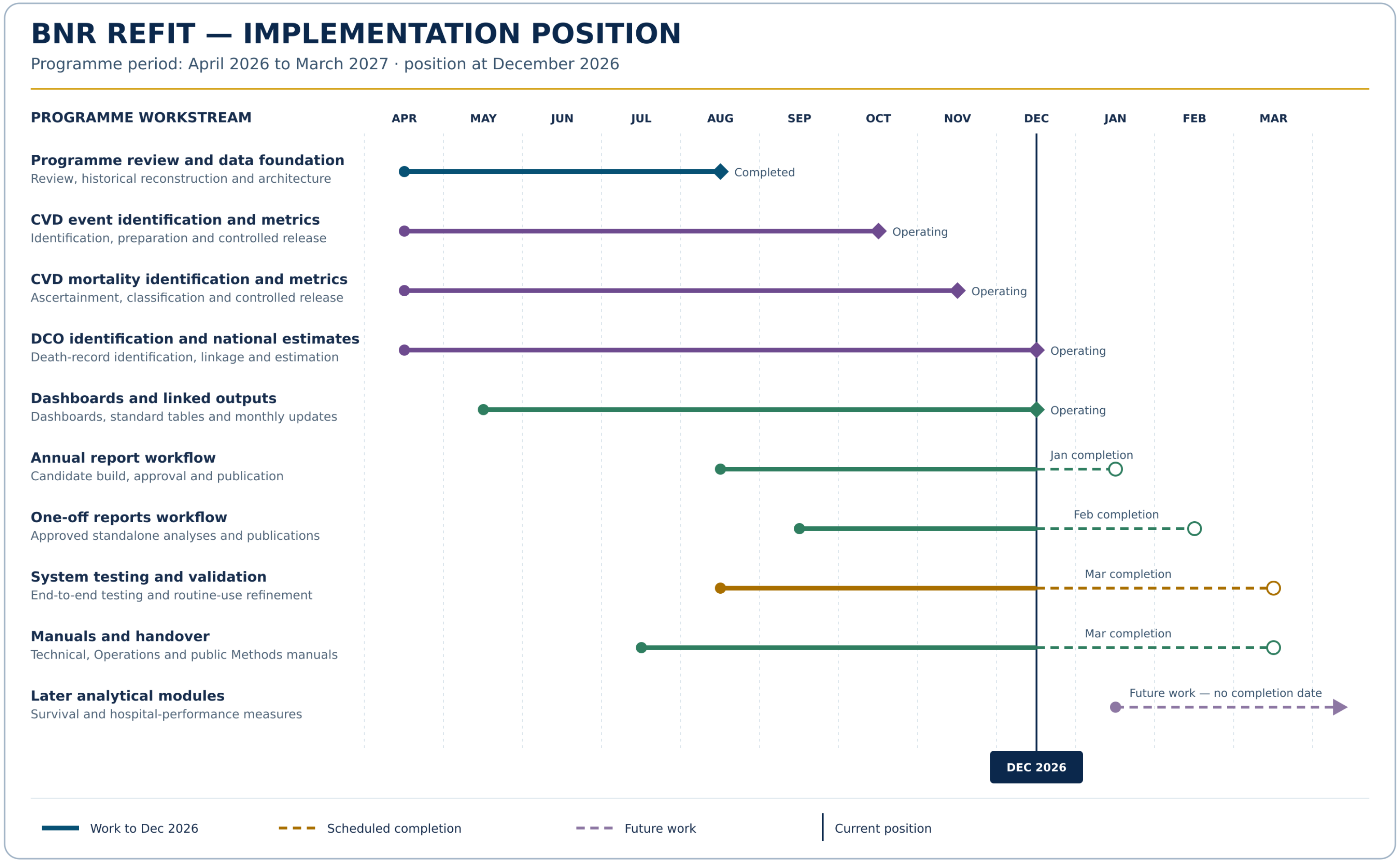
Managed data now feed separate, controlled analytics and publication workflows.

WHAT IS OPERATING

Core event, mortality, DCO and dashboard outputs were operating by December 2026.

WHAT COMES NEXT

Annual and one-off reporting, testing and manuals complete in early 2027; later analytics remain future work.



Programme view at December 2026. The annual report workflow is scheduled for January 2027 and the one-off report workflow for February 2027. Testing, manuals and handover continue to March.

Special chapter | The BNR Refit

DATA FOUNDATION

Managed REDCap data, historical records and named monthly releases.

DEFINITIONS AND METHODS

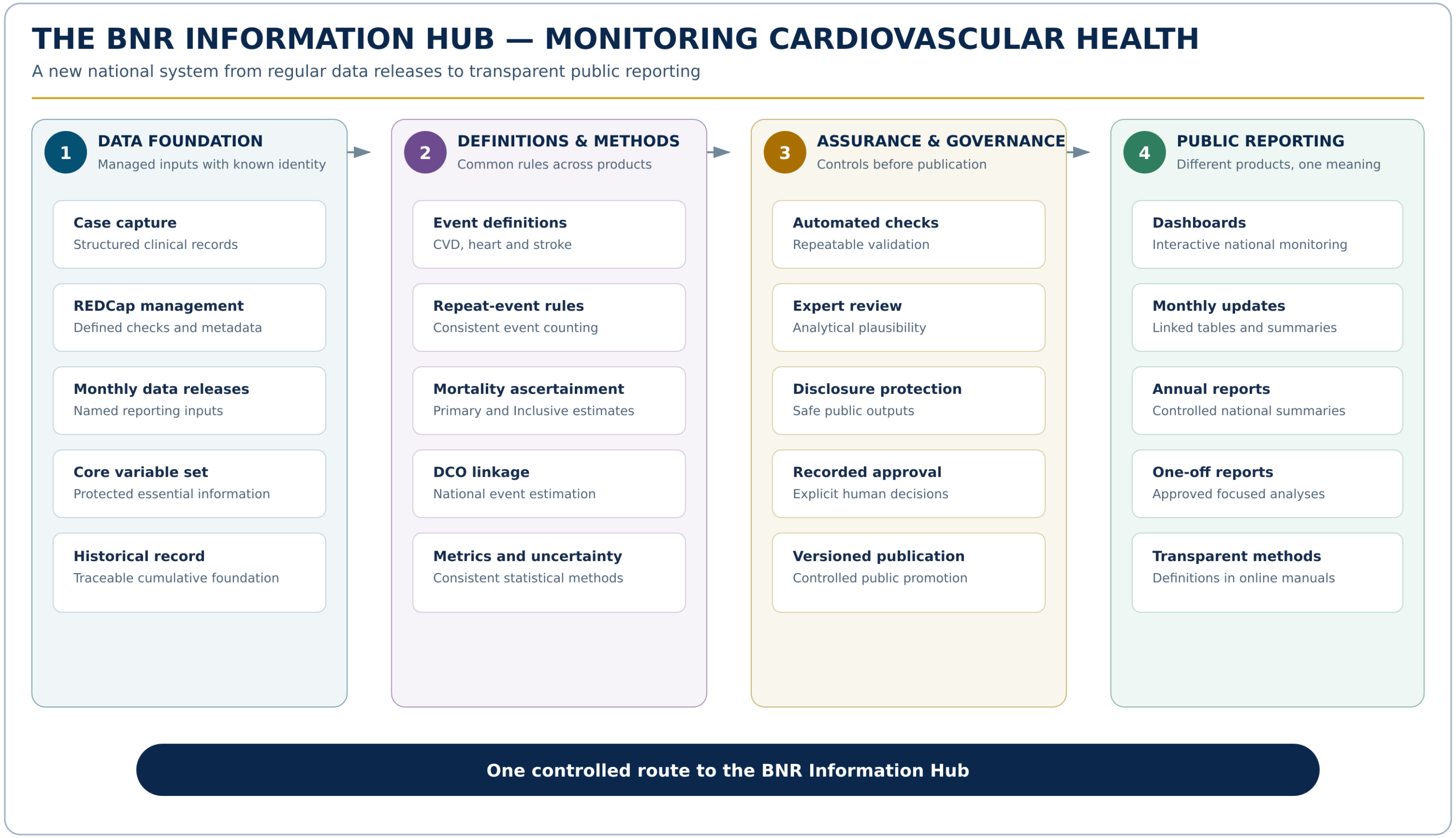
Event, mortality, DCO, metric and uncertainty methods.

ASSURANCE AND GOVERNANCE

Automated checks, human approval, disclosure protection and versioning.

PUBLIC REPORTING

Dashboards, monthly updates, reports and transparent online methods.



The four connected areas form the BNR Information Hub. Managed data use common definitions and methods, pass through explicit assurance and governance controls, and support several public products without duplicating the analytical process.

The BNR refit: current progress

The core data, event, mortality, DCO and dashboard architecture is operating. The annual and one-off report workflows will be completedin early 2027, and testing, manuals and handover continue to March. Later analytical modules remain ongoing work outside this delivery timetable.

DELIVERED AND OPERATIONAL	SCHEDULED EARLY 2027	FUTURE WORK
CVD event, mortality and DCO identification and release workflows	Annual report workflow - January 2027	Survival analysis
Dashboards with linked tables and monthly update reports	One-off report workflow - February 2027	Hospital performance measures
Controlled releases, approval and disclosure-protected publication	Testing, manuals and staff handover - March 2027	Carefully assessed new analytical modules

What this means for Barbados

1. Clearer and more timely evidence for government, hospitals, clinics and public-health teams
2. A more dependable analytical foundation with explicit dataset history
3. Consistent statistics across public datasets, dashboards, updates and annual reports
4. Transparent presentation of statistical, linkage and classification uncertainty
5. Repeatable disclosure protection and visible human approval before publication
6. A documented system designed for sustainable operation by a small BNR team

THE PRACTICAL OBJECTIVE

The same published statistic should mean the same thing wherever it is encountered - in a dataset, dashboard, monthly update, one-off report or annual report.

Public health update | CVD in 2025

Primary national estimates from the latest complete annual results

CVD EVENTS

1,116 events

229 per 100,000

Age-standardised | 95% CI 215-244

Heart EVENTS

358 events

74 per 100,000

Age-standardised | 95% CI 66-82

Stroke EVENTS

758 events

155 per 100,000

Age-standardised | 95% CI 144-167

CVD DEATHS

368 deaths

76 per 100,000

Age-standardised | 95% CI 68-85

Heart DEATHS

186 deaths

39 per 100,000

Age-standardised | 95% CI 34-46

Stroke DEATHS

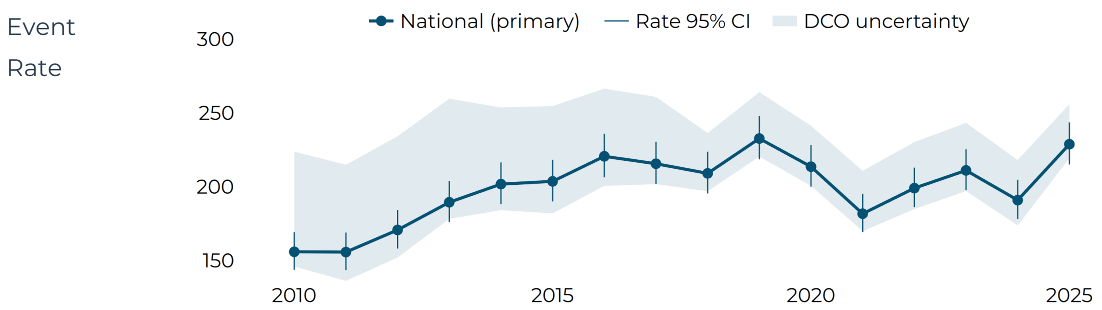
182 deaths

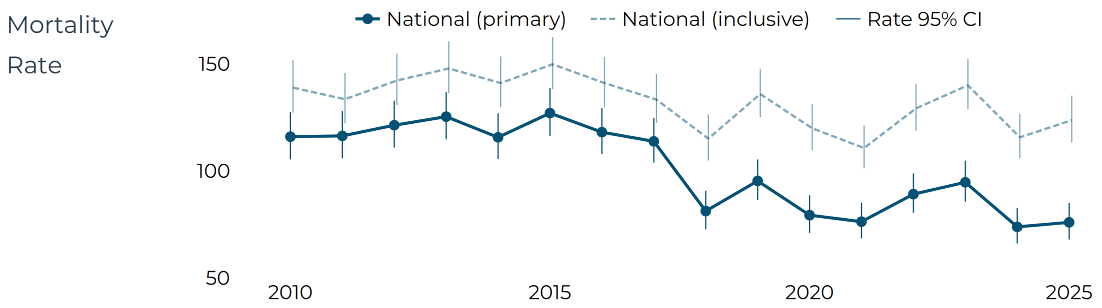
37 per 100,000

Age-standardised | 95% CI 31-43

National CVD event and mortality rate trends

Age-standardised national rates between 2010 and 2025. The event rate shading represents uncertainty linked to the identification of death certificate only (DCO) events. The inclusive mortality estimate includes deaths for which cardiovascular disease was a possible, but less certain, cause.





Key messages

01 BNR recorded 1,116 CVD events in 2025, an age-standardised rate of 229 per 100,000. The rate returned to approximately its 2019 level after several years of lower recorded rates, reinforcing the need for sustained cardiovascular prevention and service preparedness.

02 Stroke accounted for 758 events—around two-thirds of all CVD events—and its rate was more than twice the heart-event rate. However, heart disease and stroke contributed almost equally to CVD deaths, supporting strong prevention, acute care and rehabilitation pathways for both conditions.

03 CVD events returned to a high level in 2025, while the mortality rate remained near its recent low. This pattern is consistent with improved survival, although better identification of non-fatal events may also contribute. If sustained, it means more people living with CVD and greater need for rehabilitation, secondary prevention and continuing care.

Source: BNR approved CVD-event release cvd_2026_01 and mortality release mort_2026_07. Rates are age-standardised and presented per 100,000. Methods: https://uwi-bnr.github.io/info-hub/methods/